CHAPTER 11

Groups in Community Settings

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CHAPTER LEARNING OBJECTIVES

1. Describe approaches to group formation, including recruiting, screening, and selecting members (CACREP, 2016, Standard E)
2. Explain the value of group work with women
3. Explain the value of group work with men
4. Describe key aspects of group treatment associated with domestic violence
5. Understand the rationale for group treatment of people with substance use disorders
6. Identify attitudes and skills needed in working effectively with older adults in groups
7. Explore practical and professional considerations in group work with older adults
8. Explore the therapeutic value of grief work in groups

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Introduction

LO1

This chapter deals with adult groups designed by various practitioners to meet specific needs in the community agency settings in which they work, oftentimes providing affordable group services to underserved client populations. The group proposals illustrate ways practitioners have applied the concepts discussed in this book to various client populations in a community. We hope these group proposals will help you to think creatively about how to design groups to effectively meet the needs of your diverse client populations.

Among the group proposals are a women's support group for survivors of incest, a men's group in a community agency setting, a domestic violence group, and a group for people with substance use disorders. In addition, the value of group work with older adults in various community settings is highlighted in three proposals: a group on successful aging, an older adult bereavement group, and a group treatment program for institutionalized older people.

Group treatment is the preferred approach in many community agencies devoted to providing services for diverse client populations with a wide range of problems. Group work practitioners also must have an understanding of the functioning of agency systems, including how the agency is structured, policies and procedures that affect clients and treatment staff, political issues, and the existing needs of both the clients and the agency. Group work enables community agencies to offer cost-effective and clinically appropriate therapeutic services typically serving a critical need that otherwise would not be addressed due to limitations on funding in many mental health agencies. Counselors must understand the many types of client populations seen within agency settings to design effective group treatment programs tailored to meet the specific needs of these clients.

Group Work With Women

LO2

Although groups for women are as diverse as the women who comprise them, they share a common theme in their support for the experience of women. Members learn that they are not alone, and they share and begin to critically explore the messages they have internalized about their self-worth and their place in society. A group can provide women with a social network, decrease feelings of isolation, foster a sense of universality, and create an environment that encourages sharing of experiences. Kees and Leech (2014) state that groups help women to understand the systemic origins of many of their concerns and can be instrumental in changing oppressive environments in which they work and live. They believe that group members come to realize their worth and their ability to give back to each other through shared experiences, wisdom, and courage. "Groups provide women with support, hope, and empowerment to overcome adversity and they provide knowledge and education to help improve women's individual situations in life" (p. 518).

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Among the many advantages for women who choose to be in a women's group are discovering their personal strengths and resources, working on interpersonal relationships in the group context, eliminating patriarchal oppression, practicing and modeling new behaviors in a safe environment, and finding their "voice." In the group environment the members often find their voice to express their concerns, fears, secrets, and dreams. The dominant voice in a patriarchal society is based on independence, autonomy, and aloneness, and women learn early on that connection is undervalued. They may believe that what they have to contribute is not valued. In a group, women do not risk being taken for granted, for all voices are valued and encouraged.

Another advantage of women's groups is the opportunity to construct a gender analysis of what it means to be female in a patriarchal society. The gender analysis aids women in becoming aware of the external causes of their pain and struggles. It helps women differentiate between the external and the internal causes of the concerns they bring to a group. The power of the group suggests to its members that both personal change and societal change are possible.

GROUP PROPOSAL

A Women's Support Group for Survivors of Incest

The following section is written from the perspective of Lupe Alle-Corliss, LCSW. (For further information about this group, contact Lupe at her private practice; telephone: 909-920-1850; email: LupeLCSW1@aol.com.)

Sexual abuse of children by family members continues to come to the attention of mental health professionals. A sexual encounter with a trusted family member not only typically results in a major psychological trauma itself but also frequently leads to emotional problems for the survivor later in life. Some of the common problems include impaired self-esteem, negative identity formation, difficulty in intimate relationships, sexual dysfunction, and repeated victimization (Gerrity, 2014; Herman, 1992).

Various types of groups are being used in treating survivors of childhood sexual abuse including support groups, psychoeducational groups, time-limited groups, long-term or open-ended groups, and retreats (Courtois, 2010). This group proposal describes a time-limited therapy group designed to enable women to begin the process of working through unresolved issues related to their incestuous past. Today, with the rapidly changing state of mental health care, being able to conduct time-limited groups is even more advantageous because it is cost-effective, efficient, and fits the demands for short-term treatment.

Organizing the Group

The literature reveals that group therapy for survivors of childhood sexual abuse is effective and cost-effective (see Gerrity, 2014). For survivors of childhood sexual abuse, a combination of group and individual therapy may be more effective than either treatment alone (Lubin, 2007). Briere (1996) recommends concurrent group and individual therapy due to the stress resulting from clients' own memories and from hearing the stories of other group members. Structure is an important dimension in group treatment because it provides safety and allows the group members to observe consistent and clear boundaries in the therapeutic process (Gerrity, 2014). In providing a safe and therapeutic environment for incest survivors, the main goal is to empower these women by helping them get past the molestation and

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the "victimization" role. Victims can become not only survivors but thrivers if they are able to successfully work through their past experience with incest. Other objectives are to help women share their secret and recognize that they are not alone, understand the current impact of this experience, begin to work through and resolve feelings associated with their trauma, and make changes. In a group situation many women find a commonality and a basis for identification, and a new type of family can emerge, one that is different from the client's original family, which may have been dysfunctional.

I recruit potential members by publicizing my groups within my own agency by way of memos, announcements, and personal contact with colleagues. In considering membership, I seek clients who display a readiness to deal openly with the trauma of incest. Typically, these women are in individual therapy. For women who are involved with another therapist, I ask for a release form so I can consult and coordinate with the individual therapist.

Although both nonfamilial rape and incest are very traumatic, women who have experienced incest from a family member have an additional layer of trauma to work through because of the betrayal of trust. I meet regularly with potential members to assess their readiness and the appropriateness of a group experience for them and to orient them to the group process and goals. Through screening I also determine how well they are likely to fit with other potential group members. Some prospective members verbalize their eagerness to be part of the group, yet they may not actually be ready. They might be extremely uncomfortable with certain questions regarding the incest that are sure to come up in the group. Others may have been referred, and it is important to assess their true motivation because they may not be completely ready to involve themselves in this type of intense group. They may be acting on the basis of pressure or the need to please others. Also, based on the emerging composition of the group, it might become clear that a particular client may not be a compatible member in a specific group.

Clients are asked about their interest in such a group, and an attempt is made to determine how ready the individual is to talk in a group setting about the incest and its impact. Other screening questions include the following: "If you have had any previous group experience, what was this like for you?" "Were you, or are you now, involved in individual therapy, and what was this experience like?" "What are your personal goals for the group?" "What are your expectations, hopes, and fears about participating in the group?" Applicants are also encouraged to ask me questions about the group and my therapeutic style.

It is critical that members possess the ego strength to deal with the material that will be explored during the sessions. Members need to have adequate interpersonal skills to deal with others in a group situation. People with suicidal and extremely self-destructive tendencies and people who do not have adequate contact with reality are screened out. Also, clients with recent drug addictions may be at a vulnerable place in their recovery process and therefore may not be ready to enter into such an emotionally demanding group. Care is taken not to include family members or friends in the same group. Also excluded are those who are not ready or willing to talk openly about their experiences.

Group Format

The group is closed and meets weekly for 75 minutes during a 12-week period. This time limitation is designed to facilitate bonding and to produce a reasonable degree of pressure necessary to work through the members' resistances. Although each group has its own process, these groups generally go through the phases described here.

Initial Phase

The initial phase involves getting to know one another, establishing ground rules, and identifying personal goals. This stage is crucial in the development of

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trust and rapport and in giving members permission for catharsis and ventilation. A signed contract is used to encourage commitment and to help the clients feel that their participation is valued and important.

At the first group meeting I emphasize the importance of regular attendance, being prompt, confidentiality, the limitations of time, and bringing any unresolved issues back to the group rather than dealing with them outside of the group. A date for the postgroup meeting, which is typically about 3 months after termination, is established at the first session. In the early phase of the group, members express empathy with one another over the difficulty of sharing the incest issue. The following guideline questions are provided to help them deal directly with the impact of incest on them: "How did the molestation happen?" "Who molested you?" "How old were you?" "How long did it go on?" "How did you deal with it?" "How did you feel toward the people who were in a position to protect you from the molestation but failed to do so?" "What impact does this experience have on you today?"

Much of the initial phase is focused on identifying and discussing personal goals, which enables all in the group to know of each person's goals and provides a direction for the sessions. Members generally feel much anxiety and apprehension at first. A member often feels that she is the only one with such a terrible burden, and she may feel that she would be an outcast if others knew about her secret. As members realize that they have a common experience, they begin to open up and find the support available in the group useful. By sharing the incest experience, members free themselves to look at how it continues to have an impact on them. The focus of the sessions is not merely on reporting the details of the specific acts but also on exploring their feelings, beliefs, and perceptions about what happened. Although it is important for these women to share their past history, the focus in this group is on helping them deal with the effects the incest continues to have on their lives. They need to acquire present coping skills that will empower them and enable them to move forward.

Middle Phase

During the middle phase, the focus is on accomplishing the goals of individual members. Connections are made between a woman's past behavior and her present behavior. In this way she begins to see patterns and to understand her own dynamics. For instance, a woman might have chosen men who dominated her, abused her, or in some way took advantage of her. She sees with greater clarity her own part in allowing this type of treatment to continue. A group is a good way to help such a woman become aware of and challenge her faulty belief systems. Through the group process, these women can rid themselves of destructive beliefs and can learn to create functional self-statements.

As well as gaining insight into her own dynamics, a woman learns that she can be of help to others through her disclosures. By expressing intense emotions and personal sharing, a safe climate is created that facilitates further explorations of common themes such as isolation, secrecy, shame, powerlessness, hurt, and anger. This mutual sharing tends to increase cohesion within the group.

As the group evolves, other themes in an individual's life are typically identified. A number of therapeutic strategies can promote change in feelings, attitudes, and behaviors. Some specific techniques include providing reassurance for women that they behaved normally in an abnormal situation; reading books in a personal way; keeping a diary or journal that includes thoughts, feelings, and behaviors in certain situations; writing letters that are not sent; talking to other family members; and recording and sharing dreams.

A useful way for helping these women express their emotions is through artwork. By asking them to draw the abused child within, insights unfold surrounding their ordeal. Participating in the expressive arts often enables members to face the total impact of their molestation

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and to come to an emotional understanding of their past experiences. The therapeutic value of expressive arts for healing and social change is being increasingly recognized (Rogers, 2011).

Members are encouraged to continue to work on recovering memories and dealing with flashbacks, which may seem very real and are a common occurrence for the incest survivor. It is important to remind these women that they are no longer little children and that these flashbacks are only a memory of the past. Much needed support and validation is provided, and clients are assisted in recognizing maladaptive defense patterns and in developing healthier coping styles. Cognitive behavioral techniques and visual exercises also facilitate positive change.

Final Phase

Toward the end of the group the women are reminded of the upcoming termination. I assist members in reviewing what happened in the group as a whole as well as what they individually learned and how they can now continue to apply their insights and newly acquired behaviors to situations outside of the group. Role playing helps in this consolidation process. I give a structured questionnaire to help members pull together and assess their learning. The members evaluate their progress and determine future plans, including what work they still need to do. They give feedback to one another, and they identify certain people in the group who can serve as a support system. Although the women cognitively know that the group will be ending, it is a common reaction for them to not want to terminate.

Members celebrate the ending of the group by sharing food as well as participating in a "Personal Poster" in which each member can give individual support and constructive feedback to every group member. The rationale for this exercise is that each member is able to take with her some constructive feedback or supportive comments and can use this experience for support and encouragement. The poster serves as a reminder of their gains through the group experience. The personal poster exercise tends to be affirming, and group leaders can benefit from participating because doing this kind of work can be emotionally taxing. I also encourage the women in my groups to remain open to further therapeutic work if they are not already concurrently in individual or group therapy.

Group Outcomes

Building follow-up procedures into the group design provides a basis for understanding the longer-term value of the group experience as well as improving the design for future groups. In the short-term groups reported on by Herman and Schatzow (1984), the results of a 6-month follow-up survey of 28 women supported their assumption that this therapeutic approach was particularly effective in resolving the issues of shame, secrecy, and stigmatization associated with incest. Outcomes included improved self-esteem, increased safety, and decreased shame, guilt, and isolation. The single most helpful factor was the contact with other survivors.

I schedule several follow-up meetings to help group members make the transition from the weekly groups to being on their own and relying on the support networks they have developed. Part of the purpose of this follow-up is to reinforce what was learned and to provide renewed support. Follow-up sessions are scheduled 6 to 12 weeks after the end of the group. During these sessions, clients are asked these questions:

- What did you like best about the group?
- What did you like least about the group?
- How do you feel that you have changed as a result of being in this group?

Group members are also asked to rate themselves as "better," "worse," or "the same" in regard to the following areas: work, friendships, relationships with family members, intimate relationships, feelings about sex, feelings about oneself, and the ability to protect and take care of oneself.

Based on many follow-up group meetings with the members of my groups, it is clear that a well-developed

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group format with proper screening can result in a therapeutic group experience. Overall, I continue to find that carefully planned groups greatly enhance the treatment of incest survivors because they can be seen on a regular basis and can be provided with continuity and the support they need for healing. The women in these groups are able to develop a strong support network that provides them with the strength and courage to begin to resolve past issues, overcome negative patterns, and set healthy goals for their future. Perhaps the greatest message they receive is that they "deserve" to feel good about themselves and lead more productive lives. Through the years, group members have felt safe to return for brief treatment if they feel a need for further consultation.

LO3

Group Work With Men

An increasing number of men are giving expression to both masculine and feminine dimensions of their personalities. However, many men in our society still live according to a traditional masculine model of what it is to be a man. Some men are caught in rigid roles, and they may be sanctioned if they deviate from those roles or display characteristics that are not associated with their gender. Men may be so involved in their roles that they become alienated from themselves. They no longer know what they are like within because they put so much energy into maintaining an acceptable image. There is usually a price to pay for being restricted by and living by traditional male roles, especially for those men who are not in agreement with what is now considered to be truly "masculine" in our society. Men may pay a steeper price than their female counterparts for engaging in gender-atypical behaviors, which encourages men to remain deeply entrenched in masculine roles. Some men become so involved in their roles that they become alienated from themselves and no longer feel connected with their inner selves. Regardless of cultural background, gender roles that men have incorporated from cultural conditioning need to be understood and challenged if men are to make choices about aspects of their masculine identity. The challenge for men is to define who they want to be for themselves, whether that involves conforming to or rejecting traditional gender roles.

An emerging counseling emphasis on *positive masculinity* presents a more optimistic picture of changes taking place in gender roles. Kiselica and Englar-Carlson (2010) contend that positive masculinity should be given center-stage status in counseling boys and men and in conducting psychological research. Working from a framework of positive masculinity, Englar-Carlson and Kiselica (2013) transcend stereotypes and emphasize men's existing strengths; focus on men's capacities and resources; identify the qualities that empower men; view men for who they are and who they can become rather than who they are not; recognize what is right with men rather than dwelling on what it wrong with them; and shift attention to the parts of men that are good, kind, creative, successful, and capable.

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Traditional forms of individual therapy may not be the best way to reach male clients. Groups for men offer some unique advantages in assisting men in clarifying their gender roles, helping them cope with life's struggles, and in developing a sense of positive masculinity. Rabinowitz and Cochran (2002) describe how men's groups are able to deepen a man's experiences. In such a group, men are given the opportunity to face and express their disappointments and losses. Rather than denying their psychic pain and wounding, men are provided with a context where they can bring all of their feelings into the open and where they can be healed by the support of others in the group. Themes and issues that often emerge in group work with men include trust, vulnerability, fear, shame, strength, weakness, male–male relationships, competition, family-of-origin issues, sexuality, friendship, dominance, submissiveness, love, hatred, dreams, grief, obsessions, work, and death. In a men's group, members learn a great deal about themselves by sharing their experiences. One powerful intervention is the leader modeling appropriate self-disclosure by sharing some of his own life experiences.

All-male groups provide men with the support they need to become aware of the restrictive rules and roles they may have lived by and provide them with the strength to question the mandate of the masculine role. Men's groups provide a place for connecting with other men, a place to be heard, and a place to talk about being a father, having a father, relationships, divorce, aging, and transitions (Englar-Carlson, 2014). Rabinowitz (2014) states that a major benefit of participating in a men's group is the acceptance, validation, and support that each man receives. For some men, it may be the first time they have been able to reveal past experiences and traumas with the support of other men. The men's group provides the intimate connection that many men desire but often do not receive in their lives.

Most men's groups contain both a psychoeducational component and an interpersonal, process-oriented dimension. The following proposal describes such a group that one of our colleagues has facilitated for about 25 years in a large health maintenance organization in a community agency.

GROUP PROPOSAL

A Men's Group in a Community Agency

The following section is written from the perspective of Randy Alle-Corliss, LCSW, who works in a large health maintenance organization (HMO). (For further information about this men's group, contact Randy via email at RandyLCSW@aol.com.)

This proposal describes a group aimed at helping men explore ways they experience and express their gender roles. It contains both a psychoeducational component and an interpersonal, process-oriented dimension. The purpose of this group is to provide men coming to a psychiatric counseling center with an opportunity to work together on common issues such as depression, stress, marital and relationship difficulties, parenting concerns, work-related issues, loneliness, and isolation.

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Organizing the Group

Although no men's groups were being offered in our area, my coleader and I believed that men could profit from talking about life struggles in a safe and supportive group. We assumed that exploring deeply personal concerns would lead to a reduction of isolation and increase the bonding among men. Many of our male clients did not have significant male friendships; if they had friends, they were often distant from them.

Because we were working for a large HMO, we knew the number of group sessions would be limited. The group format, whereby more clients could be seen in a shorter amount of time, at a greater frequency, could serve the needs of the clients and at the same time serve the needs of the organization. We were convinced that the group would have other therapeutic benefits in addition to the cost-effectiveness factor. We assumed that the group would offer a place for men to discover universal themes, to ventilate feelings they had stored up, and to practice skills they could carry outside of the group and apply in various settings in everyday life.

We began by writing a proposal and presenting it to the administration. We developed a memorandum describing the purpose of the group and sent it to other clinicians, asking them to recommend appropriate candidates for this type of group. We excluded from the group candidates who were psychotic or suicidal, in extreme crisis, and those who lacked the psychological strength required for sustained group interaction.

Group Format

The group meets for 90 minutes each week for a total of 16 weeks. In addition, the coleaders meet regularly for 15 minutes before each session and 15 minutes afterward. The group is topically oriented, educationally and therapeutically focused, and combines a variety of techniques. At times it takes longer than one session to cover a particular topic.

The Initial Session

We start the group with general introductions from each member and ourselves. We also cover the basic ground rules in the first meeting, including confidentiality, attendance, and basic HMO policies. During this first meeting, we explore gender-related issues such as what it means to be a man, the messages the men received growing up, and how these messages affect them today. We typically discuss a number of norms pertaining to being a man. The members are invited to share their reactions to adhering to or breaking these norms and how these norms influence their daily behavior. Typical topics discussed at other sessions are described next.

Relationships With Father and Mother

We examine relationships with parents, particularly with fathers, because this seems to be a central influence in the lives of most men. Many of the men are disappointed by their father's absence or are angry about the excessive force and aggression their fathers used to discipline them. We also explore their relationships with their mothers, especially as it relates to how they handled their feelings around them. We typically invite members to write letters to their parents, which they share in the group.

Relationships With Significant Others

It is clear to us that the men in our groups often experience difficulty simply recognizing, let alone expressing, their feelings. One reason some men join the group is because they have trouble expressing feelings and behaving assertively. Members have opportunities to talk about their relationships with significant others, especially with women. Some of them may be going through a divorce or having marital difficulties. We broadened our description of relationships to include gay relationships. Our experience in working with some gay men in these groups has taught us the importance of using language that is free of bias with respect to sexual orientation. Regardless of one's sexual orientation, the men in our groups discover that they have a great deal in common.

Developing and Maintaining Friendships

We include discussion about friendships with other men and women, for we continue to find that men

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receive conflicting messages about appropriate gender roles. We also find that men have difficulty initiating friendships and maintaining intimacy. They tend to be quite isolated from other men, and one purpose of the group is to provide them with rich sources of support. We encourage the men in the group to get together outside of the group and to develop other sources of support, especially with other men. However, we caution the members about subgrouping in a way that could detract from group cohesion.

Relationships With Children

We typically examine relationships with children, as many of the men enter the group with concerns about being an effective parent. We discuss the importance of men's relationships with their children. We teach assertiveness skills to assist the men in setting limits and following through on consequences with their children. We often have the men write letters to their children, which they read in group, to help them express and explore unrecognized feelings.

Relationship to Work

Those members who have lost a job often struggle with feeling devastated over not working. The members talk about how work affects their lives at home, especially with their partners and children. Men often struggle with working too much, setting limits, and frustration on the job. Many men report feeling overly responsible and stressed when assuming the role of provider. By talking about the importance of work, many of the men learn that there is more to life than simply providing and working and come to realize the importance of a balance between work and play.

Sexuality

Most of the men have a host of concerns regarding sexuality, but they tend to be reluctant to discuss such concerns openly. We have small handouts to spur the men's thinking on particular topics, such as sexual performance, feelings of attraction to other people, erectile dysfunction, differences in sexual appetites, and aging. We have been surprised to find that men in the group felt safe enough to discuss matters pertaining to sexual abuse experiences at early ages. Due to the commonalities among the members, men can explore fears regarding their sexuality and sexual practices in ways that enable them to gain a deeper understanding of their sexuality.

Closing the Group

We end the group with good-byes and reminiscing among members and leaders. Valuable feedback is given, and the members evaluate their experience. We reinforce any gains the members have made and give them tangible items to take with them to remind them of their participation in the group. We have given out certificates of attendance with the statement: "It is okay to be a man." We encourage the members to continue their new behaviors. We also point out to them potential pitfalls they are likely to encounter in applying what they learned in the group to everyday living.

Group Outcomes

It is critical to assist the members in evaluating the impact of the group experience. Toward this end, we arrange for a postsession group, which we call the "reunion group." Between the ending of a 16-week group and the beginning of a new group, we have the members from the previous group meet again. Reminders are sent to all the members urging them to attend. The reunion group gives members a chance to take another look at what the group meant to them. We place special emphasis on how they have been applying their learning in their everyday lives. The men have an opportunity to discuss any problems they have encountered in implementing a new style of behavior. In addition to providing support, we also challenge the members to take further risks where it seems appropriate. The follow-up group gives these men a way to establish a plan for putting their new goals into action now that the group has ended.

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The members' evaluations of their experience at the follow-up session are typically positive and constructive. Many of the men say they had been looking for this kind of group for years. They often report that they greatly valued the opportunity to discuss personally significant topics in a group setting. They report feeling less depressed, less isolated, and being more able to recognize and communicate their feelings. Some men return to the group later when their benefits allow them the opportunity for continuing their treatment.

Men report that the group has helped them to manage their anger more effectively, to develop more male friends, and to become more assertive. Many of the men are able to use the feedback they receive in immediate ways by becoming more aware of their feelings and more communicative in their relationships. They begin to think of themselves and other men more positively. In general, they say that they feel more content with themselves, they feel a wider range of emotions, and they are able to laugh and have more fun.

LO4

Group Treatment of Domestic Violence Offenders

Group work is well suited for treating male offenders because of the therapeutic factors operating in a group. These therapeutic factors, discussed in detail in Chapter 8, can be helpful in addressing abusive behavior patterns. Specifically, increased universality, cohesion, and interpersonal learning are potent factors in the reduction of group members' domestic violence recidivism (Waldo, Kerne, & Kerne, 2007). The efficacy of a group approach in treating domestic violence offenders is supported by research for some groups (Lee, Sebold, & Uken, 2003).

In *Solution-Focused Treatment of Domestic Violence Offenders: Accountability for Change,* Lee and colleagues (2003) describe a treatment program that created effective, positive change in domestic violence offenders. This approach has a recidivism rate of 16.7% and a completion rate of 92.9%. More traditional approaches typically generate recidivism rates between 40% and 60% and completion rates of less than 50% (Lee et al., 2003). The approach focuses on holding offenders accountable and responsible for building solutions rather than emphasizing their problems and deficits, which is dramatically different from the traditional approach to this problem. This approach is time limited and is brief when measured against traditional program standards, consisting of only eight sessions over a 10- to 12-week period.

The solution-focused approach strongly emphasizes formation of concrete, achievable, behavior-specific goals that each participant must establish by the third session and must continue to consistently work on throughout the treatment process. The group facilitators use changes associated with each group member's goal to assist him in redefining who he is as an individual, a family member, and a community member.

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For more information on this topic, useful resources are Edleson and Tolman (1994), Lee, Sebold, and Uken (2003), Schwartz and Waldo (2004), and Wexler (2000, 2004, 2005, 2006). Wexler's 2000 book describes a well-researched program for domestic violence treatment that contains worksheets, exercises, and theoretical background for group leaders. The following group proposal describes another treatment approach with domestic violence offenders.

GROUP PROPOSAL

An Involuntary Domestic Violence Group

This section is written from the perspective of Paul Jacobson, a licensed marriage and family therapist. (For further information on this group, contact Paul at P. O. Box 1931, Willits, CA 95490; telephone: 707-513-5313; email: 4paulj@gmail.com.)

Organizing the Group

Most group counseling is aimed toward people seeking help, but the domestic violence group I describe here is for a specialized involuntary population. The men who come to this group are ordered into treatment by court judges or probation officers. Most of these group members typically arrive with attitudes of resentment, blaming others, and being convinced that they do not need counseling. The techniques and strategies for working with this type of resistance differ somewhat from those used with a voluntary population.

Resistance to Group Work

An individual assessment interview is conducted for all members prior to attending any group meetings. This often reduces their potential for disruptive hostility and can ease them into the more stressful group situation. During this time, an attempt is made to support the individual, explore the circumstances of the abusive behavior and his arrest, challenge his defenses in a moderate manner, and gain his cooperation through suggestions of therapeutic need.

The assessment interview contributes to developing a therapeutic relationship. Without establishing an honest relationship between the members and facilitator, self-disclosure in a group of guarded strangers is very unlikely. The design of this group is based on the assumption that, when given the opportunity, participants will apply personal information to themselves, even without initially accepting responsibility for their actions.

Men who batter often begin group counseling in a state of denial. They typically externalize the source of their problems, and they rarely attribute their problems to their violent behavior. For change to occur, these men need to understand their own dynamics and take responsibility for their behavior. If they do not eventually recognize and accept their own problematic behavior, it is unlikely they will change. Nor will they understand the impact of their behavior on their partners, their relationships, and themselves.

A Case Illustration

To give a concrete idea of how this resistance influences a member's participation in a group, let's consider Jerome's case. Jerome arrived for this meeting 20 minutes late, complaining that he had totally forgotten the appointment he made 2 days earlier. Jerome, like all the group members, had been ordered into the program following plea bargaining in which he agreed to attend counseling rather than spend 45 days in jail. His charges had been reduced from felony spousal abuse to a misdemeanor.

Although Jerome had chosen the group alternative, he insisted that he did not need counseling and preferred the quicker remedy of jail. He blamed the public defender for getting him into this program rather than pursuing his choice. As we talked, his initial agitation diminished somewhat. But when he described the situation of abuse with his wife, he again become loud,

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animated, and emotional. Jerome told about coming home late one night after having a few beers and getting angry because his wife had not made him dinner. When he ordered her to do so, she responded with "Make it yourself." With this he apparently lost control and claimed he was not aware of what he was doing, but he insisted he never hit her with a closed fist. Jerome was adamant that if she had only made dinner none of this would have happened. Thus, it was her fault.

In exploring other anger experiences, Jerome admitted that it bothered him a little when he became violent, and he wanted to control himself better. This statement was one of the first signs of a more personal stake for participating in the group. Further questioning found Jerome glad to be working again after several months of unemployment, and he admitted that his drinking had increased a little while he was unemployed. Jerome was clearly removed from his emotions, unaware of his own stress level, and unaware of his dependency on alcohol. Furthermore, he did not take responsibility for his actions or his feelings.

Suggestions for Leading a Domestic Violence Group

- Know your motivation for organizing and facilitating a domestic violence group.
- Be aware of your own stress level and reactions; take steps to prevent burnout.
- Try to maintain equanimity and respond rationally.
- Learn to verbalize theoretical ideas in common everyday language.
- Remember that commonly accepted therapeutic concepts are not universally understood or accepted.
- Discover your own similar issues with group members.
- Expect resistance and avoid taking it personally. Respect resistance and work with it therapeutically by encouraging clients to explore any hesitation or cautions about participating in the group.
- Use any medium at your disposal to teach and intervene (such as art, video, and handouts).
- Learn to recognize small incremental changes as progress toward discrete, obtainable therapeutic goals.
- Develop a good support system for yourself.

Group Goals

Some goals are agreed on during the assessment interview, but most are selected by the leader for individual treatment plans. Here is a list of some group goals:

- Develop behavioral alternatives to physical violence.
- Learn cognitive techniques for anger management.
- Discuss abusive events and behavioral alternatives.
- Gain an overview of relationship dynamics, expectations, and the development of physical abuse.
- Learn about physiological reactions as correlates of stress and stress reduction methods.
- Develop a recognition of feelings and emotional processing.
- Increase self-disclosures, personal discussion, and assertive communications.
- Increase personal responsibility.
- Increase empathy and understanding of significant others.
- Stop or decrease alcohol/drug use and recognize use patterns.

Group Format

Each group session begins with a didactic presentation. The group is designed to address both educational and therapeutic goals. Following the didactic segment of the initial session, members are given a written multiple-choice questionnaire. This allows individual responses without the pressures inherent with group verbalization. Then a go-around is used to stimulate discussion. Verbal participation is one of the facilitator's main goals for each group member.

The 15-week program is conducted using an open group format, and participants are added intermittently throughout the group's duration. The major advantage of an open group is to maintain an adequate number of members in the face of high dropout rates. The main disadvantage is the difficulty in developing group cohesion and trust.

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Some Ground Rules

During the initial interview members are told of the group rule of abstinence from drugs prior to meetings. If someone arrives intoxicated, he is not allowed into the group. Two such occurrences means termination. The only other requirement is attendance, and members can be terminated for missing more than the allowable three sessions.

Developing Topics for Group Interaction

Meetings are structured around topics deemed relevant to group goals. Topic discussions are designed to educate, gain involvement, convey information, and confront issues relevant to this population. The topics presented to the groups are loosely divided into areas of interpersonal and intrapersonal experience. *Intrapersonal topics* include education concerning human physiology and endocrine systems functioning, cognitive techniques for emotional processing, relaxation methods, drug and alcohol effects, stress and stress reduction, and anger. The *interpersonal topics* involving relationships include communication theories, problem-solving techniques, sex roles, relational expectations, human needs and differences, values, and family experiences.

Group Outcomes

Some of the group goals are broad and more suited for long-term treatment, yet many members show observable progress. Others continue to blame their partners for their situation throughout the group. For me, working with this population is difficult, trying, frustrating, and yet also rewarding. To remain personally invested, it becomes necessary to notice small incremental changes and regard these as signs of success and progress. One source of reinforcement is through observing improvement. I am continually forced to evaluate my statements, style, countertransference, and motivations. Developing this program helped me to examine my theoretical assumptions and assess my therapeutic skills.

Looking Back on This Group

Designing this group was a creative exercise in which I applied commonsense principles I'd learned through my most significant college classes and had applied to group work with other populations. I tried to anticipate what could be helpful with this population, and it was meaningful to develop a group that served this unmet need in my community.

If I could replay facilitating this group I would now use the research-driven handouts, worksheets, and questionnaires developed by Wexler (2000, 2004, 2005, 2006) and other more complete anger management programs for both education and feedback purposes. It is possible to obtain a more accurate picture of what works if you use worksheets at the initial, midgroup, and later stages to evaluate members' progress. For further information, see Morgan, Romani, and Gross (2014).

LO5

Group Treatment for People With Substance Use Disorders

As the problems of drug and alcohol use increase in our society, so does the demand for treatment of people with substance use disorders. Group work is the treatment of choice for dealing with substance use issues. Substance use treatment groups are usually open and have revolving membership. It is likely that some

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members will be entering treatment as others are "graduating" and ending this phase of their treatment.

Different types of groups meet clients' needs during the various phases of the recovery process (Substance Abuse and Mental Health Services Administration, 2012). *Psychoeducational groups* provide accurate information on the harmful effects of drugs and alcohol on the brain and body, withdrawal syndromes associated with specific drugs, cravings and relapse triggers, and the role of support groups in the recovery process. *Skills groups* combine an educational component with role-playing activities so clients can practice assertiveness skills, relapse prevention, and effective communication. *Cognitive behavioral groups* help clients become aware of the connection between their thoughts, feelings, and behaviors in making choices that will support their drug-free lifestyle. Some topics include cognitive errors, challenging faulty beliefs, and learning effective ways of thinking. Cognitive behavioral groups are especially important in relapse-prevention groups as clients practice making and maintaining the changes in their lives without their drug of choice. It is common for individuals to terminate treatment because they believe they have their issues under control or because they relapse into active use.

Treatment in the field of chemical dependency counseling tends to be highly structured. Substance abuse treatment is broken into stages containing structural components that may not be needed when working with other populations. Most intensive outpatient programming services are delivered in the group format and may include a weekly 2-hour group focused on events of the past week. Aftercare groups are also common. These groups provide drop-in services for graduates of treatment programs to offer ongoing support as clients maintain their new drug-free lifestyle. Education, skills training, and interpersonal process analysis may be an integral part of these aftercare groups. From detoxification through aftercare, group work provides support and helps remove the sense of isolation throughout the recovery process.

Especially in the early stages of recovery, the focus of treatment is on abstinence, stabilization, and learning skills necessary to progress in the recovery process. Most early stage drug and alcohol treatment groups are focused on assisting individuals in staying alive and living life "one day at a time." Long-term quality of life issues may be addressed but are often secondary to the tasks of restructuring life while abstaining from one's substance of choice and developing new ways of living.

Most human service agencies that work with individuals struggling with substance abuse encourage drug-free connections and sober supports outside of the formal treatment process. Twelve-step support and self-help groups such as Alcoholics Anonymous (AA) and Narcotics Anonymous (NA) are not considered treatment, but they often supplement group treatments. AA and NA are the most widely known support groups working with substance using and abusing individuals. Having an outside support system in place during treatment helps in the transition from formal treatment to termination to living a drug-free lifestyle.

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A Substance Use Disorder Treatment Group Using the Payoff Matrix

This section is written from the perspective of Kathy A. Elson, LPCC-S, LICDC-CS, SAP. She works in the direct service of treatment for individuals diagnosed with substance use disorders and is an associate professor of Human Services and Behavioral Health at Sinclair Community College in Dayton, Ohio. (For further information about this group, contact Kathy Elson; telephone: 937-512-5332; email: Kathy.Elson@sinclair.edu.)

Organization of the Group

Group treatment is the treatment of choice for dealing with addictive issues (Substance Abuse and Mental Health Services Administration, 2012.) Working with individuals in drug and alcohol treatment requires flexibility because most groups are open, with some members getting ready to "graduate" and others just starting treatment. Individuals tend to have their own level of awareness and are often in different stages of change. Members in substance use groups may be court referred, other referred, and sometimes are self-referred.

Residential treatment is an intensive level of care that is necessary for individuals who have demonstrated an inability to successfully work a recovery program on an outpatient basis. The group described is part of a 90-day residential program for individuals who have been diagnosed with substance use disorders (SUDs). In this group, the Payoff Matrix is used as an ongoing tool for change.

The Payoff Matrix has tremendous benefits for clients during their stay in residential treatment, and it is an effective tool in assisting clients in the recovery process (Mueser, Noordsy, Drake, & Fox, 2003). It is especially useful in facilitating development of insight and awareness of one's use and the consequences incurred. Members learn to make informed choices and begin to predict the consequences of their choices. An overall goal during this group activity is to assist clients in increasing their level of motivation, which is helpful in sustaining the recovery process.

Group Structure

This is an open group held for 8 consecutive weeks in a 90-day residential treatment center. It is comprised of individuals who have been formally assessed and diagnosed with a substance use disorder.

- Group consists of up to 20 members.
- Members participate weekly for 8 weeks during weeks 4 through 11 of their 13-week stay.
- Sessions last 2 hours with a 10-minute break after the first 50 to 60 minutes.
- Group sessions begin with introductions, a check-in, and a brief discussion of previous homework assignments.
- Topics for discussion include communication skills, drug and alcohol use, saying "no," relapse prevention, and changing friends. The main focus is on the underlying concept of the advantages and disadvantages of the identified topic.
- Once a topic is identified, members write down their thoughts and experiences related to the topic. The Payoff Matrix sheet is provided to the members at each group session for use in the session and to take with them for homework.
- The group utilizes individual and small group activities and large group discussions.
- Time for questions and answers is allotted.
- Group ends with a check out and an assignment focused on writing about new awareness or changes in thinking or behaviors that resulted from this experience.

Basic Group Rules

This group is held in a residential setting, and all of the rules for the residential program apply. Specific rules for this group include:

- Be on time and stay the entire group session. Return from breaks on time.
- Homework must be complete upon arrival to the session.
- Participation is expected within large and small groups.
- Be respectful of members and facilitators.
- Focus on yourself and listen to others when they speak.

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Format of the Group

Individuals need a few weeks to acclimate themselves to the residential treatment process. In week 3 of residential care, the Payoff Matrix is introduced to clients during a psychoeducational group. Prospective group members are given information about the purpose of the exercise and how it is utilized to facilitate change and decision making. Members of the Payoff Matrix group begin their participation in week 4 of their residential care.

In early recovery clients tend to be ambivalent about change. These individuals often have made frequent attempts to change but have been unable to sustain these changes. They begin by evaluating and understanding their behavior and their symptoms. They are in the process of evaluating the pros and cons of their behavior and of making behavioral changes. Although clients are often distressed by the thought of change, they do want to gain a sense of control and confidence in their ability to change.

The Payoff Matrix assists clients in identifying the possible advantages of using substances, the possible disadvantages of using, the possible advantages of refraining from use, and the possible disadvantages of refraining from use (Mueser et al., 2003). During this activity, group members are asked to provide thoughts, beliefs, and ideas consistent with the quadrant under discussion. This tool can be used for other behavioral issues as well.

The way the quadrants are organized is important. I work to determine what the main "buy-in" is for the clients in that particular group; that is, the behavior in which most members are invested. The matrix is constructed starting in the upper left quadrant with the advantages of continuing that particular behavior. The upper right quadrant is designed to record the disadvantages of refraining from the same behavior. This is followed by completing, in the lower left quadrant, the disadvantages of continuing the behavior and, finally, recording the advantages of refraining from the behavior in the lower right quadrant.

Advantages of Continuing the Behavior	Disadvantages of Refraining from the Behavior
Disadvantages of Continuing the Behavior	Advantages of Refraining from the Behavior

This order is purposeful and gives clients a chance to share the reasons they have continued a problematic behavior despite being confronted by others about the consequences of their behavior. This activity engages the clients who are typically unmotivated or are new in the treatment process.

After an initial check-in, group members review homework from the previous group session. Members are asked to participate in the Payoff Matrix activity. One example is use of drugs and alcohol. Other topics may include going to meetings or adhering to probation requirements. It is important that any beliefs the group members provide be included within the quadrants that are the focus of attention at that time, or are redirected to be included in a different quadrant. Facilitators must deal effectively with clients' resistance related to the behavior, display a nonjudgmental attitude, and be comfortable with answers that may be perceived as rough or controversial. There is also an opportunity for facilitators to ask open-ended questions and to clarify issues they want to highlight. For example, when a group member identifies "losing my home" as one disadvantage of using drugs and alcohol, I may ask this person to explain further what that experience was like. Also, if a client reports "improved relationship with children" as an advantage of discontinuing use, I may prompt him or her to keep talking about what that was like. These are opportunities for the skilled clinician to assist the client in building awareness and insight, which we hope will increase the client's motivation toward change. It is important to fill all four areas of the matrix prior to the first 10-minute break so members have time to process what was written. After the break, I start by asking if anyone has anything else they want to include in the quadrants. Once the advantages and disadvantages are listed, we open the floor for discussion.

Peer-to-peer interactions are important in a group. Although I guide the discussions, the most helpful interventions come from the members themselves. Members are notified when there are 5 minutes left for the discussion, and they are asked to think about what behavior they will use to complete a Payoff Matrix as a homework

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assignment. They can choose the topic discussed in the group that day or something related to it. Directions for the homework are given before the group closes. Group members complete a Payoff Matrix on the topic they chose and bring this homework to the next session. The results of the Payoff Matrix can also be discussed in individual sessions with primary therapists to assist in identifying areas that need further attention.

Group Outcomes

The Payoff Matrix group is a small part of the recovery process in a 90-day residential program. However, it plays a large part in identifying barriers to successful abstinence and recovery. The ultimate goal of this group is to increase levels of motivation to change. Although the main goal is abstinence and the main change is from *use* to *no use*, many other areas of life also require change to support the new behaviors. Discussing the realities of choices and consequences systematically assists clients in moving toward their goals. Awareness gained during this group process is helpful to the client and primary clinician because this insight can be the groundwork for investigating stagnant areas and to support areas of positive change and increased motivation.

There is a popular saying in the field of substance use counseling: "another tool for my toolbox." We assist clients in collecting tools that support recovery, and they can utilize these tools during their lifelong recovery process.

LO6

Group Work With Older Adults

We now turn our attention to topics related to designing and facilitating groups for older adults in community settings. Group work with older adults is one way to promote the positive aspects of aging and to help participants cope with the developmental tasks of aging. Older people have a range of life experiences and personal strengths that are often overlooked. Counselors need to develop special programs for older people and to continue their efforts to find the means to reach this clientele. Group workers will increasingly be held accountable for developing programs to help older individuals find meaning in their lives and be productive after retirement. As mental health professionals become involved with older people, their challenge is to do more than add years to a person's life—they must help individuals lead fuller and better lives.

We begin by identifying attitudes, knowledge, and skills required of those who are interested in conducting groups with older adults. Then we offer suggestions for those of you who are interested in doing group work with older adults.

Attitudes, Knowledge, and Skills of Leaders

Just as specialized knowledge is required for therapists who work with children and adolescents, so too is specialized knowledge required to treat the unique problems that older adults face. Mental health practitioners need to know more about working with older adults because this is one of the fastest growing

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groups in the United States (Kampfe, 2015), and clinicians are faced with the challenge of providing effective psychological services to this age group. Both individual and group therapy seem to be effective in treating the psychological problems of older adults. However, as with younger people, differences in culture, race, gender, sexual orientation, and social class need to be understood in making interventions with older people (APA Working Group on the Older Adult, 1998).

Christensen and colleagues (2006) found that group leaders who facilitate reminiscence groups increase their effectiveness when they utilize a slower pace, are more flexible with group process issues, are more accepting of potential distractions, and are more understanding of differing worldviews. Findings of Christensen and her colleagues indicate the importance of leaders providing the space and time for members to share their stories. Although specialized knowledge and training are required, many of the skills and methods used in leading reminiscence groups are similar to those used in a variety of different groups and settings and with diverse populations.

Your range of life experiences, as well as your basic personality characteristics, can either help or hinder you in your work. (This would be a good time to review the personal characteristics of effective group leaders discussed in Chapter 2.) We consider the following to be important assets for group work with older people:

- Genuine respect
- Positive experiences with older people
- A deep sense of caring
- Respect for the person's cultural values
- An understanding of how the individual's cultural background continues to influence present attitudes and behaviors
- An ability and desire to learn from older people
- The conviction that the last years of life can be both challenging and rewarding
- Patience, especially with repetition of stories
- Knowledge of the special physiological, psychological, spiritual, and social needs of older people
- Sensitivity to the burdens and anxieties of older people
- The ability to get older people to challenge many of the myths about old age
- A willingness to touch or be touched, if doing so is culturally appropriate
- A healthy attitude regarding one's own aging
- A background in the pathology of aging
- The ability to deal with extreme feelings of loss, depression, isolation, hopelessness, grief, hostility, and despair
- A working knowledge of the special skills needed for group work with older adults

Preparing Yourself to Work With Older Adults

If you are interested in working with older adults, you can gain valuable experience by becoming involved with older people and their families. It is important that you explore your feelings about responsibilities toward older family

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members, which can help you understand the struggles of the members of the groups you lead. Examine your attitudes about aging and challenge your negative views about older people. This self-evaluation is likely to be a lifelong task that involves reflection, supervision, training, and perhaps personal counseling (Kampfe, 2015). Here are a number of other steps you can take to better prepare yourself to work with this population:

- Take courses dealing with the problems of older people.
- Get involved in fieldwork and internship experiences working with older individuals.
- Visit agencies for the care of older adults, both in your own country and on any trips you take abroad.
- Attend conventions on gerontology, a mushrooming field.
- Investigate institutes and special workshops that provide training in leading groups for older adults.
- Become aware of your judgments or stereotypes against people based on perceiving them as old. Explore your feelings toward your own aging and toward older people in your life.
- Visit some homes for older people that represent a particular cultural or religious group, which will give you some insight into how old people are perceived and treated by different groups.
- Create a list of what you would like to see for yourself when you reach older adulthood. How would you like to be viewed? How would you like to be attended to? How do you hope you could perceive yourself? If you can identify a list of traits and qualities you hope you will have as an older person, you may get some idea of the needs of older people in your groups.
- You can learn a great deal by talking with older people about their experiences. Based on your discussions with them, create a needs assessment as a basis for a group that you may be interested in designing.

LO7

Practical and Professional Considerations for Group Work With Older Adults

Guidelines for the Group Process

Refer to Part Two of this book, pertaining to stages in the development of a group, as you begin to think about designing and conducting groups for older people in a community setting. This section provides brief examples of practical issues for you to think about in forming specialized groups for older people.

The Group Proposal It is especially important to develop a sound proposal because you may encounter resistance from community agencies that deal with older people. Include practical considerations regarding the size, duration, setting, and techniques to be used based on the level of functioning of your target group. Refer to Chapter 5 for the specific elements that might be included in a proposal for groups for older adults.

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Screening and Selection Issues The needs of older people are diverse. Carefully consider the purposes of the group in determining who will or will not benefit from the experience. Older people generally need a clear, organized explanation of the specific purposes of a group and why they can benefit from it. It is important to present a positive approach to potential group members. The anxiety level may be high among a group with older adults, which calls for a clear structure and repetition of the goals and procedures of the group. The decision to include or exclude members must be made appropriately and sensitively. For example, to mix regressed patients (such as people with Alzheimer's disease) with relatively well-functioning older people is to invite fragmentation. There may be a rationale for excluding people who are highly agitated, are delusional, have severe physical problems that could inhibit their benefiting from the group, or display other behaviors that are likely to be counterproductive to the group as a whole.

Confidentiality Institutional life is often not conducive to privacy. Older group members may be suspicious when they are asked to talk about themselves, and they may fear some sort of retaliation by the staff or fellow members. Exert care in defining the boundaries of confidentiality to ensure that confidences will not be broken and to provide a safe and nonthreatening environment.

Labeling and Prejudging Group Members Institutions are quick to diagnose and categorize people, and they are slow to remove such labels when they no longer fit. In working with older people, be careful not to be overly influenced by what you hear or read about a given member. Remain open to forming your own impressions and be willing to challenge any limiting labels imposed on your older adult clients.

Value Differences A good understanding of the social and cultural background of your members will enable you to work with their concerns in a sensitive way. You may be younger than the members, and this age difference may signal significant value differences. For example, a group leader is likely to assume that there is therapeutic value in discussing personal problems and conflicts openly. However, revealing personal matters may be extremely difficult for some older people because of their cultural values. Respect members' decisions to proceed at their own pace in revealing themselves.

Some Cautions Here are some *do's* and *don'ts* in your practice of group work with older adults:

- Do not treat people as if they are frail when they are not.
- Avoid keeping members busy with meaningless activities.
- Affirm the dignity, intelligence, and pride of older group members.
- Do not assume that every old person likes being called by his or her first name or "honey" or "sweetie."
- Make use of humor appropriately. Avoid laughing at your members for failing to accomplish tasks, but laugh with them when, for instance, they have created a funny poem.

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- Avoid talking to them as if they were small children, no matter how severely impaired they may be.
- Allow your members to complain, even if there is nothing you can do about their complaints. Do not burden yourself with the feeling that you should do something about all of their grievances; sometimes venting can be sufficient.
- Avoid probing for the release of strong emotions that neither you nor they can handle effectively in the group sessions.
- Determine how much you can do without feeling depleted, and find avenues for staying vital and enthusiastic.

LO8

Working With Healthy Aging People in Groups

Groups are a most effective approach in addressing developmental issues of aging and the key task of finding meaning in later life. In working with the *well* older adult, the social support mechanisms of a group help members understand the universality of their struggles. Many older people have problems coping with the aging process. They have to deal with the many losses associated with old age in addition to the pressures and conflicts that younger generations experience; they can profit from interpersonal groups or personal-growth groups that serve people of all ages. Kampfe (2015) suggests that group work is particularly appropriate with older people "both because this population often experiences loneliness and isolation and because interpersonal interaction is vital for physical and psychological well-being" (p. 21). The increasing demands of serving diverse populations in the community, as well as the mounting evidence of the value of group work with older adults, have fostered this emerging area of practice (Vacha-Haase, 2014).

Jamie Bludworth designed a group that focuses on successful aging and found the following references to be helpful: Erber, Szuchman, and Rothberg (1990, 1991), Kampfe (2015), Levy (1996), and Rowe and Kahn (1998).

GROUP PROPOSAL

A Successful Aging Group

This section is written from the perspective of Jamie Bludworth, PhD, a licensed psychologist and an instructor at Arizona State University. (For further information about this group, contact Jamie via email at Jamieblud@hotmail.com.)

Organizing the Group

During my time as a trainee (and later as a staff member) at a large hospital, I facilitated many groups for older adults: arthritis support groups, Parkinson support groups, dementia caregivers' support groups, bereavement groups, and a reminiscence group. These groups seemed to focus on what was "wrong" in members' lives. When group members were frustrated about setbacks in their lives, they often attributed their difficulties solely to the fact that they were aging, without considering other possible intervening factors.

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It seemed to me that many of the older people with whom I worked had bought into various negative myths and stereotypes about what it means to get older and that these beliefs were preventing them from leading more satisfying and socially engaged lives. I also encountered group members who did not appear to believe the stereotypes and myths about growing older and who were active and thoroughly engaged in their communities. They were an inspiration to the groups in which they participated. They were also a touchstone for me in the creation of a group for older adults focused solely on debunking the commonly held myths about aging and encouraging healthy lifestyle changes. I developed a group around the concept of successful aging, which was offered at the local senior center. This group became very popular among the guests of the center and was effective in meeting many of its stated goals.

Basic Components of the Group

Type of Group

The Successful Aging Group was a psychoeducational group with a developmental focus on the issues that older adults must confront in the challenging task of maintaining physical and mental strength as they move further along the continuum of aging. The group was held at the local senior center. Publicity for the group was in the form of flyers posted on the premises as well as advertisements in the senior center newsletter.

Population

Although the group was designed primarily for older adults (ages 65 and up), individuals of any age were encouraged to participate. Group members were especially encouraged to bring family members to the group to facilitate a deeper understanding of aging among and between family members.

Rationale and Basic Assumptions

Traditionally, older adults have been portrayed in a stereotypical manner by the American media. Examples of the doddering old grandfather or the memory lapsed, hearing impaired grandmother have been the butt of countless jokes in commercials and situation comedies. These images are but a miniscule sample of the misconceptions and overly simplified views that many people hold about older adults.

Although my personal experience with the members of my groups was informative, I consulted the research literature to gain a more thorough understanding of what might have been occurring. Moreover, having empirical support for the rationale of the proposed group served to lend credibility to my argument that a successful aging group would be beneficial for the clients served by the hospital.

Resources and Methods

The primary resource for this group was *Successful Aging* (Rowe & Kahn, 1998), which chronicles the MacArthur Foundation longitudinal study on aging begun in 1984 that includes several research projects in the domains of general medicine, psychology, neurobiology, sociology, and several other disciplines to create a "new gerontology." In the opening session I suggested that group members purchase a copy of *Successful Aging* to facilitate discussion and to encourage them to remain in the successful aging conversation outside of group. Supplemental literature, such as magazine, newspaper, and journal articles, was also utilized.

Group Goals

I set out to design a group that not only challenged members' belief systems regarding aging but also presented them with alternative information that was accurate, based in science, and of a positive and encouraging nature. I hoped that educational intervention, presented in an interactional group format, would affect a shift in group members' schemas regarding aging. I believed that if a shift in this domain could occur, then group members would be more likely to make changes in their lifestyle choices that could not only "add years to their lives but also add life to their years." The goals of the group were as follows:

- To foster an increased awareness of the existence of stereotypes and myths regarding older adults among group members.

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- For group members to gain an understanding of the ways in which belief in stereotypes or limiting thoughts have a discernable negative effect on their lives.
- To increase members' knowledge in the domains of physical, intellectual, and emotional health.
- For members to utilize knowledge acquired in the group to challenge and debunk the myths and stereotypes of aging.
- To create a social support network among members.
- For members to make informed lifestyle choices that would reduce the risks of age-related disease and disability while fostering greater engagement with life.

Group Format

The first half of each session consisted primarily of a psychoeducational presentation. The second half was more interactional in nature with group members sharing their personal reactions and experiences regarding the session topic. Although this group was limited to 12 sessions, it was open for new members throughout its duration. At the end of 12 sessions there was a break of 2 weeks and then the sessions resumed, recycling back to Session 1.

Session 1: Introduction

- Introduce the goals and nature of the group. Obtain informed consent.
- Introduce the concept of myths and stereotypes about aging. This can be done by asking group members to respond to the question: "What comes to your mind in response to the following words: elderly, aged, or older adult." The responses can be written on the board.
- Choose a few of these salient, popular myths and refute them with evidence. Engage members in discussion about the beliefs and attitudes they hold about the elderly and about the attitudes they believe others hold about them.

Session 2: Usual Aging Versus Successful Aging

- "Usual aging" describes older adults who are relatively high functioning but are also at a significant risk for disease or disability. Provide details and examples such as "syndrome X."
- "Successful aging" describes older adults who have the ability to maintain the following key behaviors: (a) low risk of diseases and disabilities, (b) high levels of physical and mental functioning, and (c) active engagement with life or social vitality.
- Explore any myths or stereotypes about these topics.

Session 3: Environment Versus Genetics

- Present evidence that explains how lifestyle choices are the key to successful aging and how many chronic illnesses may be prevented or treated through lifestyle changes. Some examples are smoking cessation, diet, and exercise.
- This session provides a foundation for the basic premise of the group, which states that lifestyle choices have a greater effect on one's experience of aging than do genetics.
- It is important to explain that lifestyle change is not an easy process.

Session 4: Detection, Treatment, and Prevention of Disease

- Develop strategies for the early detection of disease and encourage a self-monitoring process.
- Make the distinction between diseases that are primarily due to lifestyle (for example, hypertension, diabetes, lung cancer, and heart disease) and other kinds of genetically linked diseases such as Parkinson's and rheumatoid arthritis.
- Provide evidence (both empirical and anecdotal) for the importance of early detection in most diseases.
- Discuss the commonly recommended strategies for the prevention of disease (diet, exercise, and social engagement).

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Session 5: Exercise and Nutrition

- Present the latest findings on this topic and its relationship to healthy aging.
- Discuss community resources tailored to the nutritional and exercise regimens for older adults such as walking clubs, aquatic programs, and strength and balance programs.
- Provide a guest speaker who has knowledge of and can demonstrate appropriate exercise regimens for older adults.
- Make it clear that group members should consult their physicians before embarking on any lifestyle changes.

Session 6: Aging and Memory

- Present and discuss the latest information about the aging process and normal memory function.
- Provide detailed information about dementia and Alzheimer's disease with community resources such as the National Alzheimer's Association. Also provide information regarding the latest methods of treating dementia.
- Educate members about possible interaction effects of multiple medications and the implications for negative side effects in cognitive functioning.
- Present strategies for maintaining and enhancing memory and mental functioning such as mnemonics, appointment books, and calendars.
- Help to normalize members' experiences of minor memory losses, such as forgetting where their keys are or having brief difficulty remembering where the car is parked when shopping.

Session 7: Mental Health

- Present and discuss the topics of depression, anxiety, and possible mental health concerns with which group members might be faced.
- Many older people have negative attitudes about mental health issues and see depression or anxiety as a sign of weakness. It is important to provide education about such issues that presents a more balanced view.
- Discuss the environmental and developmental factors involved in mental health issues of older adults (death of loved ones, coping with disease or disability, and interactions of multiple medications).
- Provide a comprehensive list of community resources and be prepared to make referrals for members when appropriate.

Session 8: Relationships

- Discuss the connection between social engagement and overall health in later life. Research findings suggest that social support is a key element in successful aging, which is often overlooked.
- Describe various kinds of social support. It is especially important to mention the health monitoring function of a social support network, wherein friends encourage an individual to seek medical attention when that individual may not otherwise do so.
- Link this topic with any relationships that may be developing in the group.

Sessions 9–11: Consolidation of Learning and Preparation for Termination

Consolidation typically occurs through a review of material presented in earlier sessions. For example, in Session 9 we reviewed information presented in Sessions 1–3; Session 10 reviewed Sessions 4–6; and in Session 11 a review of Sessions 7 and 8 was the focus.

It is important to note, however, that the review sessions should not simply consist of the facilitator reciting information in a lecture type format. One exercise I used to make the consolidation sessions more interactive was "Successful Aging Jeopardy." In this exercise, I generated questions of varying difficulty that corresponded to the topics to be reviewed. For example, in Session 10 the categories were disease prevention, exercise and nutrition, and aging and memory. I then organized the questions within each category based on the difficulty of the question (difficult questions were worth more points, much like the television game show). The group was divided into two teams, with each team having an equal opportunity to answer a question.

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At first, members appeared to be reluctant to play this game. Nevertheless, after encouragement from me, and a little bit of coaxing by some of the more outgoing group members, they engaged in the exercise. It took only a few minutes before a friendly competition developed and the group as a whole became enthusiastic. This exercise provided an opening wherein group members could review information while simultaneously engaging in spirited interaction regarding the session topics and associated myths about aging.

The facilitator of this exercise needs to provide clear ground rules as to how the game will be played. Most of the ground rules that I insisted upon were interpersonal in nature. For example, members were not allowed to criticize a person if he or she could not answer a question correctly. The "Successful Aging Jeopardy" sessions tended to be the most energy infused of all the meetings. Although the game can be fun for the group, it is vital that the facilitator maintains the topical focus and continues to intervene in ways that encourage the deepening of group members' knowledge and awareness.

The topic of termination was introduced in Session 9 and briefly revisited in Sessions 10 and 11 with statements like the following: "Our meetings will be coming to an end in three sessions. It is important for each of you to take a look at how you would best like to use the time that we have remaining."

Session 12: Termination

Session 12 was devoted solely to termination. In this session, each member was encouraged to share what he or she experienced over the course of the 12 weeks spent together. Additionally, members were encouraged to discuss any changes they had made in their lifestyles and to brainstorm with each other about ways in which they might maintain such changes. Because of the epidemic proportions of isolation among the elderly, I placed great emphasis on fostering social interaction and support outside of the group, prevailing upon members to continue their interpersonal connections after termination.

Special Considerations

Group facilitators must be vigilant not to practice outside their area of expertise. Group members will often ask for medical advice. *Always* refer them to their physician for such advice. Beyond that, each group should end with a disclaimer that says something to this effect: "This group is designed to provide the latest information about aging. The group is not intended to provide medical or other professional advice. Always consult your physician or other medical professional before making any changes in your health-related behaviors."

The age of the facilitator also can be a factor in the development of trust and credibility with older adult group members. The difference in ages between the group members and me was quite obvious. By addressing this issue in the first session in a nondefensive manner, group members were able to trust me, and my credibility with them increased. It was also important for me to mention my qualifications to facilitate such a group. Moreover, I explained my interest in working with older adults in an open and honest manner, mentioning the importance of my grandparents in my life and my struggle to understand and cope with the declining health of my beloved grandmother. Group members seemed to accept my role as facilitator of the group even though I was less than half their age in many cases. I believe my genuine respect for them and honesty and openness about my motivation to lead the group created the space for this to occur.

Many group members had significant physical limitations, and it was vitally important to accommodate these limitations. For example, members who had hearing loss were seated closer to me so that they could hear what was being said. Other group members were encouraged to speak loudly so that everyone could hear. Space was made to accommodate people in wheelchairs. When I wrote on the board, I used large letters to assist those members who had visual difficulties. It was essential to be flexible in facilitating the safety and comfort of members with special needs.

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Group Outcomes

Several members of this group made significant lifestyle changes. One woman in her late 70s took up swimming again. She had been a competitive swimmer for many years but had stopped when she "got old." After several weeks of swimming at the YWCA she was invited to compete on a masters swim team. When she told the group, she was clearly very proud, and it appeared that she had a newfound confidence. Later, several of the group members went to cheer her on in her first competition in more than 40 years!

In similar fashion, a 92-year-old man had always wanted to write a book. Somehow he had never done it. Through the support he gained in the group (and some good old cajoling by group members), he started writing his memoir. He often gave the group updates about how many pages he had written. Toward the end of the group he read a passage aloud and was very moved by the experience, as was the rest of the group.

One woman in her 70s started a monthly outing group, organizing trips to museums, a flea market, a wine tasting, or other such events. The group chartered a small bus and took photographs of their adventures that they would later share with the rest of the group. This motivated other group members to join them. Many people in the monthly outing group reported that it had been years since they had been out with a group of people and that they felt more vital and alive from doing something that they simply had "let slip."

Many members of the group made new social connections and were able to provide support for each other outside of the group. In one instance, members of the group noticed something different about another member and encouraged her to see a doctor. This led to early detection of a potentially life-threatening disease that was treated before it became unmanageable.

Finally, the older group members were not the only beneficiaries of the successful aging group. I was deeply moved and inspired by many of the members. I saw firsthand the resiliency and dignity of older women and men who often were overlooked by society and their families alike. I learned that a sense of humor goes a long way when things get tough. Most of all, I learned the value of true mutual respect. Some of the members of those first successful aging groups have since died. Although I have been saddened by their deaths, I have also taken great solace from knowing that they were known and cared for by the members of their group and that in some cases they had taken the opportunity to reclaim something that they had lost.

The Therapeutic Value of Grief Work in Groups

LO8

Grief work, or *bereavement*, refers to the exploration of feelings generated by a significant loss. The loss of family members, friends, and intimate relationships brings out human wounds that need to be healed. After a loss, some common feelings include sadness, sorrow, fear, hurt, confusion, depression, resentment, relief, loneliness, anger, despair, shame, and guilt. *Mourning* involves recognizing our internal grief and expressing it externally. There is therapeutic value in the process of mourning, which entails dealing with grief on both an emotional and an intellectual level. Grief is a necessary and natural process after a significant loss, but far too often grief is pathologized. Unresolved grief may linger in the background, preventing people from learning to live with their losses and delaying their interest in forming new relationships. Grief may be over many types of losses besides a death, such as the

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breakup of a relationship, the loss of one's career, or children leaving home. Wolfelt (2015) recognizes the value of leaning *toward*, not *away from*, grief. If we hope to heal, we must directly experience our grief. When people deny sadness and the pain associated with loss, they inevitably suffer more in the long run and are less able to express a range of feelings. This unexpressed pain can be physically and psychologically immobilizing and can prevent accepting the reality of the death of a loved one.

Groups can be instrumental in helping people feel less isolated as they move at their own pace in their own way in attending to their pain and grief. Older adults struggling to adjust to the many changes that confront them through loss can benefit from having an opportunity to grieve and to recognize the significance of their loss. Increased loneliness is often a key factor following the death of a loved one, and a group can offer the older person much needed support and socialization (Vacha-Haase, 2014). Although addressing issues such as loss and pain in grief groups is important, the possibility of developing new relationships is critical and can be explored in these groups.

Bereavement is a particularly critical developmental task for older people, not only because of the loss of others who are close to them but also because of the loss of some of their capacities. Although death strikes at children as well as older people, facing one's own death and the death of significant others takes on special significance with aging. If people who are experiencing bereavement are able to express the full range of their thoughts and feelings, they stand a better chance of adjusting to a new environment. Indeed, part of the grief process involves making basic life changes and experiencing new growth. Group counseling can be especially helpful to people at this time.

Many forces in our society make it difficult for people to experience grief. Social norms demand a "quick cure," and other people often cannot understand why it is taking "such a long time" for a grieving person to "get back to normal." In our fast-paced world today, people are urged to "get over it and move ahead." Hedtke and Winslade (2004) provide a strikingly different perspective on working through losses and moving ahead. They believe people are born into networks of relationships and remain woven into those networks long after they die. Those who experience the loss of someone they love can find comfort in developing a new relationship with the person who died. This relational approach to grief work is based on a process of "re-membering," which is about continuing to foster the memory of a person's life even after he or she dies. By keeping this person's voice and thoughts alive, we have gained an ongoing resource that helps us move forward in life. We can consistently renew a dead person's presence in our lives by remembering the stories and imagining the voice of the loved one as we reflect on what he or she might say to us as we face difficult times. Those who lead bereavement groups can glean some very useful ideas on grief and bereavement from Hedtke and Winslade's (2004) *Re-membering Lives: Conversations With the Dying and the Bereaved.*

If you would like to learn more about designing and conducting bereavement groups with older people, Alan Forrest, who designed an older adult bereavement group, recommends Capuzzi (2003), Christensen et al. (2006), Evans and Garner (2004), Fitzgerald (1994), Freeman (2005), Hedkte and Winslade (2004), James and Friedman (1998), Tedeschi and Calhoun (1993), Wolfelt (2003, 2015), Worden (2002), and Yalom and Vinogradov (1988).

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An Older Adult Bereavement Group

This section is written from the perspective of Alan Forrest, EdD, a professor of counseling at Radford University. (For more information on this group, contact Alan at Radford University, Department of Counselor Education, P.O. Box 6994, Radford, VA 24142; telephone: 540-831-5214; email: aforrest@radford.edu.)

Introduction

Before I began facilitating loss and bereavement groups, I was filled with enthusiasm, excitement, and altruism, tempered somewhat by numerous fears, anxieties, and insecurities. At times words are inadequate to express the intensity of feelings in the group; that is how I learned the value of therapeutic silence and how one's presence can be healing. To hear and truly feel another person's pain is not without effort, and knowing you cannot take away that pain can be frustrating and depressing. The first group session is usually the most difficult because each group member relates the circumstance of his or her loss. The hurt, suffering, and pain can be overwhelming, but there is also comfort in knowing that one does not have to be alone.

With age comes an increased number of losses: friends, family, and other losses such as retirement, declining health, home, and for some, mental and cognitive functioning. All of these life transitions, in addition to losses through death, need to be grieved and can be addressed in a bereavement group. The goal of an older adult bereavement group is to facilitate movement through the stages and the tasks of grief by providing an open and supportive environment that promotes emotional recovery. Group participants are encouraged to express their needs, wants, and wishes in addressing their grief and to identify goals they would like to accomplish in the group.

Organizing the Group

One of the primary goals of a bereavement group is to educate the mourner to the reality that grief is a process that is measured in years, not months. It does not proceed the same for everyone; as one widow stated, "You never really get over the loss, but the challenge is learning how to live with it." I strive to be clear about the purpose and structure of the group and focus on creating a group culture of mutual support.

This group has a psychoeducational focus, including elements of education, emotional support, and encouragement for social interaction. The group serves as a catalyst for meeting the members' emotional needs, for dispelling myths and misconceptions of loss responses, and for enhancing the ability to develop new relationships outside the group. To achieve a high level of group cohesion and trust, I have designed a closed, time-limited group. The group meets for 8 to 10 sessions, each lasting for 2 hours, and is limited to no more than eight members.

Screening

The screening and selection process is an essential variable contributing to the success of the group. It is necessary to have a clear sense of who will likely benefit from a group in making the determination of whom to include. One consideration is how much time had passed since the individual experienced the loss. Most people whose loss had occurred less than 12 weeks prior to seeking treatment are too early into their bereavement and are not ready for a group experience. Individuals who are experiencing additional psychological, emotional, interpersonal, or crisis problems are poor candidates for this type of group and may serve to defocus the group away from exploring and working on the grieving process. It is essential to rule out candidates who exhibit serious pathology as they are likely to impede the group process and take precious time from the other group members. I have discovered that individuals who possess basic social skills and who are comfortable with self-disclosure contribute most to making the group both satisfying and beneficial.

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Counselor Self-Awareness

To function successfully as a grief counselor requires an awareness of self and one's own experiences of loss. The issues that encompass grief and loss are very emotional and touch our deepest fears as human beings. If the counselor is not aware of personal loss issues, the grief work with clients experiencing loss will be seriously affected and compromised.

The experience of bereavement necessitates that the counselor examine, at least minimally, three personal areas. First, working with bereaved individuals increases one's awareness of one's own losses. Examining personal issues (past and present) surrounding loss and bereavement is a difficult task, filled with profound sentiments. Counselors need not to have fully resolved all the loss issues they have experienced, but they should be aware and actively involved in working on them. If a client is experiencing a loss similar to that of a recent loss in the life of the counselor, it may be difficult to therapeutically help the client.

Second, grief may interfere in working with clients with regard to a counselor's own feared losses. Although all counselors have experienced losses in the past, there may be apprehension over future losses. Counselors may encounter personal anxiety when thoughts of losing their own children, partners, or parents enter into their work with clients. This is usually not problematic unless the loss a client presents is similar to the one we most fear. For example, if the counselor has an elderly parent, he or she may have difficulty facilitating an elderly bereavement group.

A third area involves the counselor's existential anxiety and personal death awareness. Death is a part of the cycle of life, and working with older adult bereaved individuals is a constant reminder of the inevitability of loss in one's own life and in the lives of those we care for deeply. Most of us think about our own mortality and, in varying degrees, experience anxiety about it. However, it is possible to acknowledge this reality and not allow it to impede our work with clients. How we choose to respond to the losses in our own lives will determine how effective we are in providing comfort and growth experiences in the lives of others.

Leadership Issues

Leadership style and the activity level of the leader are important considerations for a bereavement group. A high degree of leader involvement is effective during the early stages of group. But as the group evolves into a cohesive unit, I tend to become less active. I recommend a coleader format. One coleader can concentrate on helping an individual express and work through a painful experience while the other coleader attends to members reacting to this pain.

Disruptive behaviors can impede the group process. It is important to address disruptive behavior as soon as it occurs. For example, it is not unusual for a group member to express the attitude that "my loss is greater than your loss." I handle this by saying, "We are not here to compare losses. Everyone's loss is unique. We are here because of losing someone, and everyone's loss is important in this group." Other disruptive behaviors to be aware of include the person who habitually gives advice but rarely engages in personal sharing; the person who gives moralistic advice and uses "shoulds," "musts," and "have tos"; the member who observes but seldom participates in personal sharing; the person who makes irrelevant comments; the individual who habitually self-discloses and monopolizes the group; and the person who continually interrupts other group members when they are working on their loss issues.

Group Format

Norms and Expectations

During the initial session of the group, I discuss group norms and expectations, including beginning and ending on time and socializing outside of the group. I also address members' questions about how the group functions. Ending sessions on time can be difficult because it is hard to accurately gauge the length of

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time it takes for the expression of the intense emotions that are elicited.

Socializing outside of group is generally discouraged because subgroups may form, which can have a negative effect on the cohesion and process of the group. However, I do not adhere to this norm when leading bereavement groups for older adults. There are advantages to outside contact among members, such as helping them deal with the loneliness and social isolation that are a part of their bereavement experience.

I try to identify common themes in group members' narratives in an effort to link common experiences and feelings of the group participants. It is important that group members become aware of the similarities and common elements of their loss so they do not experience a sense of isolation and aloneness. The therapeutic factors of universality and instillation of hope are present and can serve as an incentive for group members to continue to participate in the group despite an upsurge of painful emotions.

Participant Introductions

At the first group meeting I ask participants to introduce themselves and share something personal about themselves. I monitor these disclosures so that each member has approximately equal time to share. The introductions provide each member with the opportunity to be the focal point of the group and also facilitate the universality of the feelings and expression of grief. Introductions are generally limited to name, circumstances of the loss, fears and anxieties participants have, and anything else they want others to know about them. One useful technique is to have group members share what they were thinking and feeling while they were on their way to this first meeting. The commonalities are noted, which begins a linking process. Due to the excessive anxiety many individuals experience, it is sometimes necessary to do this exercise in dyads prior to introductions to the entire group.

Sharing Personal Loss and Bereavement Experiences

At some point in the first group session each participant is given the opportunity to discuss in detail his or her loss and bereavement experiences up to this point in life. Introductions allow for some of this to occur, and as group members become more comfortable with one another, they are more receptive to discussing their losses in greater detail and in more personal ways.

Also, group members are not discouraged from talking about other concerns that they need to bring to the group. Often these other issues are related to their life as a result of their loss. A sampling of such topical concerns may include family realignments, role adjustments, finances, managing a household, relocation, and even dating once again.

Understanding the Grief Process

Perhaps the single most important gift counselors can offer bereaved individuals is an accurate understanding of the grief process so individuals no longer believe their grieving is abnormal or that they are "going crazy." Areas typically discussed are descriptions of the grief process, possible reactions such as physical symptoms, suicidal ideation, increased dependence on medication, depression, typical behaviors, and the reactions of others to one's loss.

Wolfelt (2003) describes a model of understanding grief that includes 10 "touchstones" or trail markers that one can use to navigate through the oftentimes confusing process of grief and bereavement. Wolfelt refers to the touchstones as "wisdom teachings" that can assist group members to better understand the grief process and better understand themselves. The touchstones are opening to the presence of loss, dispelling misconceptions about grief, embracing the uniqueness of grief, exploring the feelings of loss, recognizing one is not crazy, understanding the needs of mourning, nurturing oneself, reaching out to others, seeking reconciliation, and coming to appreciate one's transformation. These touchstones can be incorporated into

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the group process and can even be used as an outline for a 10-week group format.

Topical Issues

Topics generally addressed in the group include guilt, loneliness, a changing identity, fear of letting go of the deceased, interdependence, multiple losses, an increased personal death awareness, changed social and interpersonal roles, important dates such as birthdays and anniversaries, religion and spirituality, rituals, the influence of family members on the grieving process, nutrition and physical exercise, and termination of the group itself, which is another loss that participants will need to address. Although the group facilitator may suggest any of these themes for discussion, it is important to be receptive to the topics group members want to explore.

Photographs and Memories

In an effort to make the deceased real and concrete to the other group members, I encourage members to bring in and share photographs. This facilitates the reminiscing process, which may serve as a form of life review. Each group member is given the opportunity to tell the group about the person in the photograph. This exercise assists group members to feel a deeper sense of each other's loss and gives each mourner the chance to reminisce and share a memory, thereby reaffirming his or her relationship with the deceased.

Addressing Secondary Losses

Secondary losses are frequently ignored in the bereavement process despite the fact that they can be as powerful as the loss itself. A secondary loss—a loss that occurs as a consequence of a death—may take many forms and can affect a group in dramatic fashion. For example, one widow, whose husband had died very suddenly and unexpectedly shortly after taking early retirement, was struggling not only with him physically being gone but also with the loss of all the hopes, plans, and dreams they had shared. It was difficult for her to discuss moving on with her life because her future plans were so intertwined with those of her husband. It was not until she examined her secondary losses that she could open up, engage in the group, and receive group support. Clients need to identify and anticipate secondary losses so they can be prepared for and grieved.

Emotional Intensity

A bereavement group contains a high level of emotional intensity, but many clients are unable or ill prepared to deal with the breadth and depth of emotions they will encounter in themselves or in others. A degree of vulnerability accompanies the expression of emotions, and many participants may not be ready for it. Others may perceive this vulnerability as a sign of weakness. Therefore, these concerns must be addressed repeatedly throughout the group experience. Some group members will weep from the start to the end of each group session, which clearly communicates how difficult the group is for them. This emotion is acknowledged, but it may not be immediately addressed. Members may require time to process their emotions before feeling comfortable in verbally expressing them. These individuals receive support from the group by others respecting their need, for a period of time, to maintain some emotional distance.

Confidence and Skill Building

Many of the bereaved older people, particularly widows, had been very dependent on their spouses. It is possible that some of this dependency may be transferred to their adult children. Although these individuals are grieving and are in emotional pain, they have the capacity to develop new skills. Doing so can bring increased confidence and self-esteem. One group participant whose husband died disclosed that

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she was having difficulty balancing her checkbook because her husband had always attended to family financial matters. When others in the group shared similar situations of being dependent on their spouses, she was encouraged and went to her bank to learn how to balance the checkbook. This particular woman gained so much confidence in herself that by the final group session she shared that she had bought a new car, something that her husband had always done for her. It is important to remember that confidence and self-esteem are just as essential for an older adult as for anyone else.

The Use of Touch

Many older adults, particularly those who have lost a spouse, have a strong need to be touched. Without a spouse, it is difficult to get this need met. If a counselor is comfortable with the use of therapeutic touching, it is a way to meet the need of being touched on the part of the client and also a way for the counselor, or other group member, to physically make contact with another. It is paramount that the counselor is clear as to the appropriateness of the touching and whether or not the client is willing to be touched. A word of caution in using touch with a grieving individual: the touch may be countertherapeutic if it is used to communicate or is interpreted as "Don't cry, everything's going to be all right." Crying is part of the healing process, so group facilitators need to carefully attend to the participants and take their lead from them.

Religion and Spirituality

The religious and spiritual domains have often been overlooked or ignored by counselors. However, bereaved older adults need to examine their adjustment to the world, particularly to their assumptive world. A person's core beliefs and values can be shaken by losing a loved one or a valued friend. It is not uncommon for an older bereaved individual to feel that he or she has lost direction, purpose, and confidence in life. Group members often search for meaning or purpose in their life, experience a loss of a sense of connection, have feelings of guilt or unworthiness, question their faith, have a desire for forgiveness, and may experience a sense of abandonment by God. Meaning, faith, and connection with God (or a Higher Power) may be called into question when an older person responds to the death of a close loved one. The group can serve as an opportunity to examine such issues that frequently are not discussed and shared with others out of fear, shame, guilt, or embarrassment.

It is normal and reasonable to expect that many bereaved older adults who have experienced a loss are suffering, to some degree, from the anguish caused by questions of meaning, faith, or specific religious or spiritual issues. Asking questions about how people are doing spiritually, how they are coping with their loss, and listening for broader meanings can create an atmosphere of openness and receptivity to talk more about their loss from a spiritual perspective.

One group member expressed her anger at God for allowing her husband of over 40 years to be killed in an automobile accident. Although she was a woman of faith and had previously attended church on a regular basis, she expressed her anger at God and discontinued her church attendance and involvement, thereby cutting out a likely source of spiritual, social, and interpersonal support.

Termination

Some group members have terminated prematurely and suddenly after two or three group sessions. Follow-up interviews indicated a number of reasons for leaving, some of which include members' perceptions that the group is too emotionally threatening, too intimate, too sad, or that they "should" be able to cope with their loss on their own. It is important to be cognizant of generational, cultural, and gender differences when working with an older adult population. Separately, or together, each will have

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an effect on whether or not one chooses to remain in the group.

What are some lessons that can be learned from the termination of the group as a whole? Termination of the bereavement group provides participants with another opportunity to learn how to deal with loss in a healthy manner. Members are encouraged to address any unfinished business toward the deceased or toward other group members or the facilitators. Group participants are also encouraged to provide each other with feedback on their experience in the group and to say good-bye. The members can anticipate how it will be to no longer have a group, explore what the group experience has meant to them, and examine their plans for continued support.

Rituals

A ritual is a set of actions, performed primarily for symbolic value, designed to preserve the traditions of a community. Grief rituals may provide a process for people to reach a level of accommodation to a different way of living. Most cultures have prescribed rites of passage that honor transitions into the different stages of the life cycle. Bereavement rituals can serve to separate the bereaved from the rest of society and assist the bereaved in a reintegration and return to society as a changed person. Funerals and memorial services are perhaps the best known examples of bereavement rituals.

Rituals can be used effectively throughout the group experience and can be powerful tools to increase self-awareness, especially during the termination process. One technique that has proved to be a powerful group ending experience is to have each participant light a candle that represents the deceased. Participants share with the group any thoughts about what they learned or about their deceased or what the group has meant to them. This exercise engages the group in a final shared experience, provides an ending ritual, uses light as a metaphor for insight and illumination, and leaves each participant with something concrete (the candle) to take home. Group members have found this to be a highly emotional and healing experience.

Group Outcomes

How effective are older adult bereavement groups? Although no empirical outcome studies have been done on the older adult bereavement groups I have facilitated, there is some interesting anecdotal information that supports the value of these groups.

A primary goal of an older adult bereavement group is to reduce the isolation and loneliness that may be the result of a loss, particularly when someone has lost his or her spouse of many years. At the conclusion of one of the groups, all the participants exchanged phone numbers and would periodically meet informally for emotional and social support. Another group contracted with me to meet once a month, for 3 months, in what we referred to as an "aftercare group" following formal termination of the group. In each of these groups, all members had been strangers at the beginning of the group.

What have I learned from facilitating these groups? I have shared tears, and I have shared laughter working with bereaved older adult groups. It may seem paradoxical, but I have learned more about the process of living than about dying and grieving from facilitating these groups. Facilitating an older adult bereavement group is life affirming and personally enriching. I have learned to celebrate each day as a precious gift and to deeply value my personal relationships. Wisdom, courage, and strength of character cannot be taught in graduate school, but I have profited from some wise, courageous, and strong individuals—my clients.

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A Group Treatment Program for Institutionalized Older Adults

This proposal is written from the perspective of Marianne Schneider Corey.

Introduction

Many different types of groups are suitable for older people. The number and types of groups possible are limited only by the imagination of the counselor and the counselor's willingness to create groups to meet the special needs of older clients. The group described here is a preplacement group in a psychiatric facility for older people who will be returning to their home or going to another kind of residential living facility in the community.

Organizing the Group

The preplacement group consisted of individuals currently in an institutional setting who were going to be released into community settings—either returning home or going to board-and-care residential facilities. As with other group populations, it is necessary to give careful consideration to the selection of members. A group is not likely to function well if severely disturbed and hallucinating members are mixed with clients who are psychologically intact. It is a good practice, however, to combine talkative and quiet people, depressed and ebullient types, excitable with calmer clients, suspicious with more trusting individuals, and people with different backgrounds. The size of the group is determined by the level of psychological and social functioning of the participants.

The preplacement group that my coleader and I formed consisted of three men and four women—a good balance of the sexes and a workable number for two leaders. Before the first meeting I contacted the members individually and gave them a basic orientation to the group. I told them the purpose of the group, what the activities might be, and where, when, and for how long the group would meet. I let each person know that membership was voluntary. When people seemed hesitant to attend, I suggested that they come to the initial session and then decide whether they wanted to continue.

Group Goals and Group Format

Before the first session my coleader and I decided on a few general goals for the group. Our primary goal was to provide an atmosphere in which common concerns could be freely discussed and in which members could interact with one another. We wanted to provide an opportunity for members to voice complaints and to be included in a decision-making process. We felt strongly that these people could make changes and that the group process could stimulate them to do so.

The group met once a week for an hour in the visitors' room. Before each group session I contacted all the members, reminded them that the group would meet shortly, invited them to attend, and accompanied them to the group room. I learned that it was difficult for them to remember the time of the meetings, so individual assistance would be important in ensuring regular attendance. Those who were absent were either ill or involved in an activity that could not be rescheduled, such as physical therapy. The group was open; members would occasionally be discharged, and we encouraged newly admitted patients to join. This did not seem to bother the members, and it did not affect the cohesion of the group. As leaders, we also attempted to make entrance into the group as easy as possible.

We always allowed some time for the new members to be introduced and to say anything they wanted to about being new to the group, and we asked current group members to welcome them.

The Initial Stage

During the initial sessions, members showed a tendency to direct all of their statements to the two leaders. In the hope that we could break the shell that isolated each person in a private and detached world, my coleader and I immediately began to encourage the members to talk to one another, not to us. When members talked

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about a member, we asked them to speak directly to that member. When members discussed a particular problem or concern, we encouraged others to share similar difficulties.

In the beginning members avoided talking about themselves, voicing complaints, or discussing what they expected after their release from the institution. Their usual comment reflected their hopelessness: "What good will it do? No one will listen to us anyway." Our task was to teach them to listen to one another.

The Importance of Listening and Acting

My coleader and I felt that one way of teaching these people to listen would be by modeling—by demonstrating that we were really listening to them. Thus, when members spoke of problems related to life on the ward, my coleader and I became actively involved with them in solving some of these conflicts. For example, one member complained that one of the patients in his room shouted for much of the night. We were able to get the unhappy member placed in another room. When some members shared their fears about the board-and-care homes they were to be released to, we arranged to take them to several such homes so that they could make an informed choice of placement. One woman complained that her husband did not visit her enough and that, when he did take her home, he was uncaring and uninterested in her sexually. On several occasions my coleader and I held private sessions with the couple.

Some men complained that there was nothing to do, so we arranged for them to get involved in planting a garden. Another group member shared the fact that she was an artist, so we asked her to lead some people in the group and on the unit in art projects. She reacted enthusiastically and succeeded in involving several other members.

Our philosophy was to encourage the members to again become active, even in a small way, in making decisions about their lives. We learned *not* to do two things: (1) encourage a patient to participate in an activity that would be frustrating and thereby further erode an already poor self-image or (2) make promises we could not keep.

Listening to Reminiscences

In addition to dealing with the day-to-day problems of the members, we spent much time listening as they reminisced about sadness and guilt they had experienced, their many losses, the places they had lived and visited, the mistakes they had made, and so on. By remembering and actively reconstructing their past, older people can work to resolve the conflicts affecting them and decide how to use the time left to them. In addition, members enjoy remembering happy times when they were more productive and powerful than they are now.

The Use of Exercises

My coleader and I designed a variety of exercises to catalyze member interaction. We used these exercises as simple means of getting group interaction going. We always began by showing how an exercise could be done. Here are some exercises we used in this group:

- Go on an imaginary trip, and pick a couple of the other group members to accompany you. (Although you may have to deal with feelings of rejection expressed by those not chosen, this exercise is very helpful for people who are reluctant to reach out to one another and make friends.)
- If you could do anything you wanted, what would it be?
- Bring a picture of you and your family, and talk about your family.
- Describe some of the memories that are important to you.
- Tell what your favorite holiday is and what you enjoy doing on that day.

Another exercise that helped older people focus and contributed to member interaction is the sentence-completion method for low-level clients (Yalom, 1983). Incomplete sentences can be structured around a variety of themes. Here are a few examples:

- Self-disclosure (One thing about me that people would be surprised to know is . . .)
- Separation (The hardest separation that I have ever had is . . .)

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- Anger (One thing that really irritates me is . . .)
- Isolation (The time in my life when I felt most alone was . . .)
- Ward events (The fight on the ward last night made me feel . . .)
- Empathy (I feel touched by others when . . .)
- Here-and-now interactions (The person whom I am most like in this room is . . .)
- Personal change (Something I want to change about myself is . . .)
- Stress (I experience tension when . . .)

Working with incomplete sentences can trigger intense emotions, and the group leader needs to be skilled in dealing with these feelings.

By encouraging the members to express themselves, these exercises led to their getting to know one another, which led in turn to a lessening of the "what's the use" feeling that was universal in the beginning.

Debunking Myths

My coleader and I explored some myths and attitudes that prevail regarding the aged and challenged the members' acceptance of these myths. Anyone working with older people needs to differentiate between myths and facts surrounding what it means to grow old. The facts are that older people can and do learn and are able to make changes in their lives. Creative and informed professionals can do much to enhance the lives of their older clients, thereby helping to dispel the myths that have acted as a barrier to providing quality services for older adults.

Termination

I was to be coleader of this group for only 3 months, and I prepared the members for my departure several weeks in advance. After I left, my coleader continued the group by himself. On occasion I visited the group, and when I did I was remembered and I felt very welcome.

Group Outcomes

To work successfully with older adults, one must take into account the basic limitations in their resources for change yet not adopt a fatalistic attitude that will only reinforce their sense of hopelessness. Had my coleader and I expected to bring about dramatic personality changes, we would soon have been frustrated. The changes that occurred were small and came slowly. Instead, we expected to have only a modest impact, and so the subtle changes that took place were enough to give us the incentive and energy to continue. Here are some of the outcomes we observed:

- Members realized that they were not alone in experiencing problems.
- People in the group felt an acceptance of their feelings and realized that they had a right to express them.
- The group atmosphere became one of trust, caring, and friendliness.
- The members continued the socializing that had begun in the group outside group sessions.
- The members learned one another's names, which contributed to increased interaction on the ward.
- Participants engaged in activities that stimulated them rather than merely waiting for their release.
- The members began to talk more personally about their sorrows, losses, hopes, joys, memories, fears, regrets, and so on, saying that it felt good to be listened to and to talk.
- Staff members became involved in thinking of appropriate activities for different members and helped the members carry them out.

My coleader and I also encountered some frustrating circumstances during the course of the group. For instance, the members occasionally seemed lethargic. We later discovered that this occurred when the participants had received medication just before the group session. Still, it was difficult to discern whether a member's condition was due to the medication or to psychological factors. It was not uncommon to find a member functioning well one week and feeling good about herself and then to discover, the next week, that she had had a psychotic episode and was unable to respond to anyone.

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Some small changes that occurred in the group sessions were undone by the routine of residential life. Some of the members resisted making their life on the unit more pleasant for fear of giving the impression that their stay would be a long one. It was as though they were saying, "If I communicate that I like it here, you might not let me go." Other members resigned themselves to institutional life and saw the facility as their home, expressing very directly that they did not want to leave.

Working with older adults can be very rewarding, but it also can be draining and demanding. Sometimes I felt depressed, hopeless, and angry. These feelings were seldom directly connected with my activities with the patients; the smallest changes I observed in them were rewarding and gave me an incentive to go on. I felt annoyed when I saw student psychiatric technicians or other staff members show disrespect for the patients. These students often called an elderly person by his or her first name yet insisted that they themselves be addressed by their surname. Occasionally, an agitated patient would be restrained physically or with medication without ever being asked what had made him or her so upset in the first place. Another upsetting sight was a patient in a wheelchair being pushed around and not being told where he or she was going.

Sometimes the behavior of patients was treated as crazy when in reality there was a good reason for it that could have been discovered if anybody had tried. One day a student brought a blind person to the meeting, and the blind man proceeded to take off his shoes. The student shouted at him to put them back on. I approached Mr. W. and kneeled in front of him. "Mr. W., you are taking your shoes off in our group session," I said, "How come?" He apologized, saying he thought he was being taken to the physical therapy room.

In another case, a 75-year-old patient kept taking his shoes off all day long, always gathering up newspaper to put around his feet. Everyone considered this behavior bizarre. I remembered that during my childhood in Germany I was often told in the winter to put newspaper into my shoes to keep my feet warm. After spending some time with this man, I found that this "strange" behavior was based on the same experience. I learned to be careful not to judge a patient as bizarre or delusional too quickly but rather to take the time to find out whether there was a logical reason for a peculiar behavior.

Patients were sometimes discouraged or made to feel shameful when they physically expressed affection for another member on the unit. Sensuality was perceived by the staff as bad because of "what it could lead to." I had several very good discussions with staff members about our attitudes toward the sexuality of the aged. By dealing with our own attitudes, misperceptions, and fears, we were able to be more understanding and helpful to the patients.

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Points to Remember

Groups in Community Settings

Groups Designed for Community Settings

- In any kind of group for adults it is essential to be sensitive to diversity. Techniques should be appropriate for the life experiences of the members and should not be forced on members.
- A good understanding of the social, cultural, and spiritual backgrounds of your members will enable you to work with their concerns in a sensitive way.
- Groups can help adults integrate current life changes into an overall developmental perspective. In such a group the members can be encouraged to reflect on who they are, where they have been, and to identify future goals.
- Domestic violence groups and groups for people who abuse substances are increasing in prominence. Conducting such groups demands a great deal of patience and competence in dealing with difficult, often involuntary clients.
- Group workers will increasingly be challenged to develop programs to help older adults find meaning in their lives and be productive after retirement.
- Group counselors often encounter obstacles in their attempts to organize and conduct groups for older adults. Some of the barriers are due to the unique characteristics of this population, yet other obstacles are found within the system that does not support group work for older adults.
- Groups offer unique advantages for older people who have a great need to be listened to and understood. The group process encourages sharing and relating, which has a therapeutic value for the older person.



Exercises

In-Class Activities

1. **Critique Group Proposals.** After reading the various proposals for groups with adults in this chapter, which specific proposal most captures your interest? Form small discussion groups in class to analyze and critique the special types of groups for adults and older adults that are described in this chapter. As you look at each group proposal, discuss what features you find to be the most innovative, interesting, and useful. If you were going to design a similar group, what changes might you want to make from the proposal? What are you learning about designing a particular group from studying these proposals?
2. **A Group for Women.** Review the women's support group for survivors of incest described in this chapter. What do you see as the unique therapeutic factors that can promote healing in this kind of group?
3. **A Group for Men.** Review the men's group in a community agency. Would you be inclined to join this group? Why or why not?
4. **A Substance Use Disorder Group.** Review the substance use disorder group proposal. What are some unique features of this group that most interest you? If you were asked to design and facilitate a group for people with substance abuse problems, how useful would this proposal be to you?

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5. **Groups in a Community Agency.** If you worked in a community agency and were asked to organize a group for either women or men, what steps would you take in designing a men's or a women's group? Assume that you were expected to form a psychoeducational group. What topics would you build into your group with either men or women?
6. **Organizing a Group for Healthy Older People.** Review the description given of the successful aging group. What did you learn from reading this proposal? If you were asked to form a group for healthy aging people in a community center, what steps would you take to organize this group and to get members? What are some things you would want to tell potential members about this particular group?
7. **Organizing a Bereavement Group.** Review the description given of a bereavement group. What are the main elements that account for the therapeutic value of grief work? What are some advantages of using a group format for dealing with loss with older adults? If you were to form a similar group, what changes might you make in organizing and conducting such a group? What challenges do you expect to face in conducting this kind of group?
8. **Techniques for Institutional Groups.** Review the group proposal for institutionalized older people described in this chapter. What specific techniques and exercises do you think are most useful with this particular group?
9. **Community Groups for Older People.** Assume you are doing an internship or are employed in a community mental health center and have been asked to develop strategies for reaching and serving the needs of older people in the community. What steps would you take in assessing community needs and developing appropriate group programs? What kinds of personal issues might either help or hinder you in being able to effectively facilitate a group for older people?

MindTap for Counseling

Additional resources can be found on CengageBrain.com and by logging into the MindTap course created by your professor. There you will find a variety of study tools and useful resources that include quizzes, videos, interactive exercises, and more.

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